Guidelines for peri-operative steroid replacement in patients with adrenal insufficiency

Patients with adrenal insufficiency (Addison's disease/pituitary hypoadrenalism) need additional glucocorticoid doses during surgery.

These recommendations are for uncomplicated peri-operative cases. They do not replace clinical judgement, but aim to eliminate adrenal crisis and reduce risk of grossly excessive steroid dosing.

- For elective surgery in a patient with adrenal insufficiency, please inform the consultant endocrinologist responsible for their care to enable peri-operative management/advice.
- For emergency surgery, please inform the endocrinology registrar covering the wards as soon as possible. For advice out-of-hours, please contact an endocrine consultant via switchboard.
- Changes may be needed in response to BP and fluid status in individual circumstances.

	Guidelines for glucocorticoid replacement in patient with adrenal insufficiency	
	Nil by mouth e.g. prolonged nausea after surgery without sepsis or hypotension - Hydrocortisone 25mg qds IV	
	Type of Surgery	**Glucocorticoid dose §**
For Example	*Minimal*	
	<1 hr local anaesthesia Minor dental procedures Skin biopsy	Usual oral steroid dose.
	Minor	
	Herniorrhaphy Colonoscopy Dental surgery > 1hr local anaesthesia	Double dose of oral steroid on day of procedure *or* Hydrocortisone 25mg iv at induction followed by normal oral dose.
	Moderate	
	Total abdominal hysterectomy Laparoscopic cholecystectomy Joint replacement	Hydrocortisone 100mg iv at induction. Hydrocortisone 25mg iv qds for first 1-2 days (if not eating and drinking). Followed by double normal oral dose tapered to normal dose over next 3 days if well.
	Major	
	Cardiothoracic surgery Major intra-abdominal surgery	Hydrocortisone 100mg iv at induction. Hydrocortisone 50mg iv qds for first 2-3 days. Followed by double normal oral dose tapered to normal dose over next 3 days if well.
	Critical	
	Major trauma Life-threatening complication	Hydrocortisone 100mg iv immediately, followed by 50mg iv qds.
	§ Patients should take normal dose of steroid up to the point of surgery. If not first on list but NBM, give hydrocortisone 25mg IV qds until induction. Patients who have sepsis or other serious postoperative complication should revert to 50mg iv hydrocortisone qds and endocrine advice sought.	